

Tata 1mg doesn't have a traffic problem. It has a *refill* problem.

Horizon · 4 quarters
Unit of analysis · the chronic patient-month
Baseline · illustrative, ₹190 Cr / month

Monthly revenue is an identity: for each line, **MTU × orders per user × AOV × net realisation**. The instinct is to push term one with discounts and installs — at ₹1,180 AOV and low-20s product margin, that user needs five orders to pay back and most never get there. Terms two and four are leaking against patients already on the platform. **Everything below works those two.**

BASELINE / MONTH	ADDRESSABLE UPLIFT	FROM EXISTING USERS	MARGINAL CAC NEEDED
₹190 Cr Pharmacy, diagnostics, wellness and ancillary, net of discount and returns.	+₹38–40 Cr Net of overlap between levers; ₹42.8 Cr gross. About +20%.	~64% Of the uplift comes from frequency, basket and fill — no new CAC.	₹0 For levers 1, 4, 6 and 7. Acquisition is the last lever, not the first.

01 Where the revenue actually is

Monthly revenue, ₹190 Cr baseline

Pharmacy carries the volume; diagnostics and wellness carry the margin. Ancillary is the only line with no inventory behind it.

Pharmacy · ~22% margin **₹136.5 Cr** Diagnostics · ~55% GM **₹25.0 Cr** Wellness & OTC · ~35% GM **₹21.0 Cr**
Ancillary · 60–90% GM **₹7.5 Cr**

- 01 Chronic is an annuity that leaks
- A patient on metformin, glimepiride and a statin should generate twelve refills a year. Platforms capture seven or eight. A missed month isn't deferred — the patient walks to the chemist downstairs and the habit resets.
- 02 Out-of-stock is silent churn
- An unserviceable line doesn't lose its own value alone. It splits the basket: the patient needs all three strips together, so the whole order goes offline. Line fill rate is a revenue KPI wearing an ops badge.
- 03 The molecule already names the test
- Metformin implies HbA1c every ninety days; thyroxine implies TSH. Pharmacy knows the molecule and the dose date, and diagnostics carries twice the margin. Single-digit cross-penetration is the largest un-priced asset here.

The thesis: treat the chronic patient-month — not the order — as the unit of revenue. Every lever either adds a patient-month, protects one that was going to leak, or raises what a patient-month is worth.

02 The footprint — actors, purpose, jobs to be done, success metrics

ACTOR	PURPOSE	JOB TO BE DONE	SUCCESS METRICS
DEMAND SIDE — FIVE PEOPLE, FIVE DIFFERENT CLOCKS			
Chronic patient 45–70 · 3–6 molecules	Stay on therapy without a gap, at a predictable price.	"When my strip is down to four days, get me the same molecule at the same price before I miss a dose — without me having to remember."	Refill-on-time % · Dose-days covered · M6 repeat · Molecule persistence
Caregiver Ordering for a parent	Run someone else's regimen without being the failure point.	"When I manage my father's six medicines from another city, give me one profile, one reminder and one bill — and tell me when he's about to run out."	Profiles per account · Multi-profile order share · Cohort M3 retention
Acute buyer Episodic · speed-elastic	Resolve tonight's problem tonight.	"When I'm ill at 9pm, get it to me in two hours — otherwise I walk to the chemist and you lose this order and the next one."	Sub-4h serviceable pin codes · Express share · First-second order
Wellness shopper OTC & supplements	Act on a self-diagnosed goal without reading 400 labels.	"When I decide to fix my vitamin D or my gut, tell me what to take and for how long — one credible choice, not a category page."	OTC AOV · Private-label mix % · 60-day category repeat
Diagnostics user Doctor-led or preventive	Turn a doctor's instruction into a report, fast.	"When my doctor says get an HbA1c, send a phlebotomist at 7am and give me the report before my 6pm appointment."	Slot adherence % · Sample-to-report TAT · 90-day retest rate
FULFILMENT SIDE — THE CONSTRAINT ACTORS. NONE OF THEM SELL ANYTHING; ALL OF THEM CAN CAP REVENUE			
Verifying pharmacist Statutory Rx gate	Validate the prescription so the order can legally ship.	"When an Rx lands, let me clear or query it fast enough that the order doesn't die in my queue — and give me a legible image the first time."	Verification TAT · Rx rejection % · Drop-off at verification
Teleconsult doctor Partner or panel	Generate a safe, fillable prescription — and with it, demand.	"When a patient consults, give me their order history and last reports so I can prescribe safely in eight minutes."	Consult-Rx % · Rx-fill % · Repeat consult rate
Warehouse & dark-store ops Inventory placement	Have the exact SKU within delivery range of the patient who takes it.	"When demand shifts at a pin code, tell me early enough to re-depth the SKU — before a line goes short and takes the basket with it."	Line fill rate · Pin-code SKU availability · Cold chain · Inventory days

ACTOR	PURPOSE	JOB TO BE DONE	SUCCESS METRICS
Phlebotomist / lab Collection & processing	Convert a booking into a valid, on-time report.	"When I accept a 7-9am window, give me a clustered route so I actually make it, and flag fasting cases before I arrive."	Slot adherence · Sample rejection % · Collections per phlebo-day
Delivery partner Last mile	Close the loop the patient actually experiences.	"When I pick up, give me an address that resolves on the first attempt and a handover that doesn't need three calls."	On-time % · First-attempt delivery % · Cost per delivery
MONETISATION & GOVERNANCE – TWO PAY FOR TRAFFIC ALREADY BOUGHT; ONE CAN SHUT THE BUSINESS DOWN			
Pharma / FMHG brand Advertiser	Reach intent-qualified patients at the moment of brand choice.	"When a diabetic searches for my molecule, put my brand in front of the substitution decision — with creative that clears compliance."	Ad revenue per order · Inventory fill % · ROAS · Share of search
Insurer / corporate HR B2B2C channel	Deliver a health benefit to a covered population at a known cost.	"When I run a benefit for 20,000 employees, give me one vendor for checkups, pharmacy discount and utilisation data I can defend to finance."	Contracted lives · Activation % · Revenue per covered life
Regulator CDSCO · FDA · DPCO · NMC	Constrain. Not a customer; has no upside to sell.	"Ensure no Schedule-H molecule dispenses without a valid prescription, and no scheduled formulation prices above its ceiling."	Rx audit pass % · DPCO breaches = 0 · Dispensing exceptions
Tata Neu ecosystem Traffic & loyalty	Lower blended acquisition cost; give health a reason to recur.	"When a Neu user has coins to spend, make health the category they spend them in — and send that customer back to us."	Neu-sourced MTU share · Blended CAC · Cross-brand repeat

03 KPI tree

L0 the outcome · L1 line of business, because margins differ by 2× · L2 the revenue identity, the only four things that can move a line · L3 what a team can act on. Chips in orange are the drivers the plan moves.

L0 · MONTHLY NET REVENUE ₹190 Cr Revenue = $\sum_{\text{lines}}$ (MTU × Orders per user × AOV × Net realisation)	
L1 · Pharmacy 71.8% · ~22% margin ₹136.5 Cr Monthly transacting users 1.00 M New 180k · CVR 2.4% Reactivated 120k Retained 700k · M6 38% Orders per user 1.30 Refill-on-time 47% Auto-refill 4% Reminder-order 18% Dose-days covered 71% Average order value ₹1,180 Lines/order 3.4 Rx-OTC attach 22% Private label 11% 90-day pack 9% Net realisation 89.0% Discount depth 9.4% Line fill 91% Cancellation 4.8% Returns 2.1%	L1 · Diagnostics 13.2% · ~55% GM ₹25.0 Cr Monthly transacting users 220k Pharmacy→labs 6% Doctor-Rx-led 18% Corporate lives 1.4 M · act. 9% Bookings per user 1.05 Quarterly protocol adherence 21% Family-profile 27% Average order value ₹1,150 Tests/booking 2.6 Preventive packages 31% Radiology attach 4% Net realisation 94.0% Slot no-show 7% Sample rejection 1.4% Report TAT 22h
L1 · Wellness & OTC 11.1% · ~35% GM ₹21.0 Cr Monthly transacting users 350k Organic / SEO 54% Attached to an Rx order 22% Content-cart 3.1% Orders per user 1.15 60-day repeat 24% Subscription 6% Course adherence 41% Average order value ₹580 Private label 19% Units/order 2.2 Pack upsize 12% Net realisation 90.0% Discount depth 8.1% Returns 1.9%	L1 · Ancillary 3.9% · no inventory ₹7.5 Cr Retail media ₹4.2 Cr Ad revenue/order ₹25 Inventory fill 34% ROAS 4.1× Orders 1.70 M Teleconsult ₹1.1 Cr Consult-Rx 61% Rx-fill 43% A funnel, not a P&L line Offline & B2B ₹2.2 Cr Stores & centres 190 Rev/store-month ₹9.2 L Contracts 34 Care Plan subscription in pharmacy Penetration of pharmacy MTU 4% Member frequency 1.9×

L0 outcome L1 line of business L2 revenue identity term L3 operable driver orange = moved by the plan

04 Seven levers, ranked by monthly rupees

#	LEVER	KPI IT MOVES	FROM → TO	₹CR / MONTH	MARGIN	IMPACT IN
1	Chronic auto-refill engine. Model dose-days per user from pack size and directions; prompt at day 26, one-tap or standing order.	Orders per pharmacy MTU	1.30 → 1.42	+12.6	~22%	60-90 d

#	LEVER	KPI IT MOVES	FROM → TO	₹CR / MONTH	MARGIN	IMPACT IN
2	Diagnostics protocol cross-sell. Map molecule → guideline test → cadence; prompt inside the refill flow, not as a campaign.	Labs penetration of chronic base	6% → 11%	+7.9	~55%	90–120 d
3	Care Plan penetration. Price the subscription against the chronic basket — free delivery, fixed discount, lab credits — so it prepays frequency.	Subscriber share of pharmacy MTU	4% → 9%	+6.3	Prepaid	120 d
4	Rx → OTC and private-label attach. Condition-aware attach at cart (statin → CoQ10, metformin → B12), pharmacist-reviewed, capped at two.	Pharmacy AOV	₹1,180 → ₹1,225	+5.7	~45%	60 d
5	Retail media on Rx intent. Sponsored placement on molecule search and substitution, compliance review on every creative, no Schedule-H promotion.	Ad revenue per monetisable order	₹25 → ₹47	+4.1	~90%	90 d
6	Line fill rate and cart substitution. Pin-code forecasting on chronic molecules, plus a pharmacist-approved equivalent offered before the basket breaks.	Line fill rate	91% → 96%	+3.7	Blended	45–90 d
7	Personalised discount depth. Stop paying retention money to people who would have refilled anyway; concentrate depth on at-risk and first-refill cohorts.	Net realisation	89.0% → 90.5%	+2.5	Pure	30–60 d
Total, gross of overlap — levers 1, 3 and 4 double-count one patient, so net ≈ ₹38–40 Cr, about +20%			₹190 → ₹233 Cr	+42.8		4 quarters

And what should lose the same meeting. Deeper platform-wide discounting — payback runs past three quarters at this AOV and it trains the base to wait for the coupon. New-city expansion before line fill in the existing top twenty clears 95% — buying demand you cannot serve costs the order and the customer. A new vertical before the chronic engine works: every launch competes for the same product and pharmacist capacity that levers 1, 2 and 6 need.

95 Sequencing

The first thirty days buy no revenue. They buy the instrumentation without which the other eleven months are guesswork.

DAYS 0–30	DAYS 31–60	DAYS 61–90	QUARTERS 2–4
Instrument, ship the cheapest lever • Dose-days model per user per molecule — the spine of levers 1, 2 and 3 • Line fill rate by pin code, daily, owned by growth as well as ops • One chronic cohort definition shared by pharmacy and labs • Refill reminders on the top 12 molecules	Protect the basket, price the habit • Standing orders with pause, skip and pre-dispatch confirmation • Substitution at cart for short lines — equivalent brand, pharmacist-approved • Care Plan repriced around the chronic basket, sold at the second refill • Attach engine on top condition-OTC pairs, two suggestions maximum	Open the high-margin adjacencies • Protocol lab prompts to the chronic cohort, timed to the refill, booking in the same flow • Retail media beta with 6–8 brands, compliance gate on creative • Discount depth by cohort replaces the flat rate, with a held-out control	Scale what the first quarter proved • Auto-refill beyond the top 12; refill-on-time toward 58% • Omnichannel: stores and collection centres as same-day fulfilment nodes • Corporate and insurer channel, priced per covered life, activation owned • Private label depth where attach already proved demand
Guardrails — Rx compliance audit pass = 100% DPCO ceiling breaches = 0 Contribution margin / order ↑ or flat Complaints per 1,000 orders Delivery SLA adherence Unsubscribe rate on refill nudges Attach-driven return rate Pharmacist verification TAT			

On the numbers. The ₹190 Cr baseline and every L2/L3 value are **illustrative** — a consistent model built so the structure is legible and the arithmetic checkable, not a report of Tata 1mg's actual figures, which are not public at this granularity. Replace them with actuals and the tree, the levers and the sequencing all hold; only the sizing moves. Cross-check first: orders per MTU against your own chronic cohort, line fill rate at the line rather than the order level, and net realisation after returns rather than after discount alone.